JAMA Network Open

Invited Commentary | Substance Use and Addiction

Overdose and Homelessness—Why We Need to Talk About Housing

Kelly M. Doran, MD, MHS; Callan Elswick Fockele, MD, MS; Marcella Maguire, PhD

In their cohort study, Fine et al[1] found that drug overdose mortality among adults experiencing homelessness in Boston increased substantially from 2004 to 2018, with a particularly rapid growth in deaths that involved multiple drugs or synthetic opioids. The cohort's standardized mortality rate from overdose was 12 times higher than that for the adult population of Massachusetts.[1] Other studies that were conducted elsewhere in the US have reported similar findings.[2-4] In Philadelphia, for example, drug overdose has been the leading cause of death among people experiencing homelessness since at least 2011, with the number of such deaths doubling in a 4-year period.[4]

Fine et al[1] highlighted the frequently overlooked association between drug overdose death and homelessness. People experiencing homelessness are marginalized, often literally as witnessed in encampment clearings or sweeps. Federal funding favors research on pharmacologic treatments for addiction over research on interventions targeting the social and structural determinants of health, including housing. Consequently, many researchers and addiction practitioners may feel they are ill-equipped to address the seemingly insurmountable problem of homelessness.

The number of people experiencing homelessness, and particularly unsheltered homelessness, has been growing in the US since 2016, after several years of declines. At the national point-in-time count of homelessness in January 2020 (before the COVID-19 pandemic), there were more than 580 000 people living in shelters or unsheltered (eg, on the streets or in encampments).[5] Although representing the best available data, such counts underestimate the true extent of homelessness, in part because homelessness is often a transient or episodic state that may not be captured at a single point in time. For example, people may move between homelessness, institutions (eg, jails and hospitals), and staying with family or friends. Moreover, millions of people experience housing instability that is short of literal homelessness. People who use drugs are at heightened risk, with 1 study reporting that approximately one-third of US addiction program clients had recently experienced homelessness.[6] Studies in other subpopulations of people who use drugs have reported similarly high homelessness rates.[7]

A common assumption is that there is a unidirectional causal pathway between drug use and homelessness; that is, people become homeless because of their drug use. This misconception places the blame on the individuals and away from the root structural contributors to homelessness as well as perpetuates stigma and points to the wrong solutions. In reality, the association between homelessness and drug use is bidirectional, and homelessness itself plays a role in drug use and overdose risk. People experiencing homelessness may use drugs for adaptive reasons, such as to stay awake or to sleep as needed or simply to adjust and cope with the trauma of homelessness. The criminalization of both homelessness and drug use compels potentially dangerous behaviors, such as using drugs hastily in public locations or alone in hidden spots. Depending on the locality, people experiencing homelessness may be arrested for something as basic as sitting down in the wrong location. As individuals cycle in and out of jail, they face disruptions in treatment and forced periods of nonuse, which may be followed by drug use upon release and thus increased risk for overdose. Constant movement, as induced by frequent street or encampment sweeps, makes connecting to resources and supports even more difficult.

Fine et al[1] emphasized the need to expand evidence-based treatment and overdose prevention strategies for people experiencing homelessness who use drugs, including supervised injection sites, naloxone distribution, safer drug supply, and substance use treatment offered in both nontraditional and office-based settings. Such interventions are important, particularly when tailored to meet the

+ Related article

Author affiliations and article information are listed at the end of this article.

Open Access. This is an open access article distributed under the terms of the CC-BY License.

Downloaded from jamanetwork.com by guest on 12/07/2025

needs of people experiencing homelessness, to fill key gaps in the substance use treatment ecosystem. However, none of these interventions address the root cause of homelessness, which is lack of affordable housing. Because structural factors drive homelessness, interventions must include not only addiction treatment but also affordable housing, living wages, and a stronger social safety net. Furthermore, Fine et al[1] found that the relative difference in overdose mortality between people experiencing homelessness and the general population was substantially larger for White individuals compared with Black and Latinx individuals. This finding highlights that structural racism may disproportionately contribute to homelessness among Black and Latinx communities, even in the absence of individual factors such as substance use disorder or mental illness. Nationally, overdose rates among Black individuals have accelerated in recent years.[8] Addressing the racial and ethnic disparities in both homelessness and drug overdose requires countering structural racism, including thoroughly enforcing fair housing policies and redressing long-standing inequities in housing and health opportunities.

In our efforts to stem the tide of overdose deaths, we should not treat homelessness as a foregone conclusion. Homelessness is not inevitable or unactionable. The solutions to homelessness are known, evidence-based, and widely accepted, although they are not yet brought to scale. Rental subsidies (ie, Housing Choice Voucher Program, also known as Section 8, vouchers) are effective in durably resolving and preventing homelessness.[9] Historically, the number of available housing vouchers has been far lower than the need, but this situation may be improving somewhat with new federal investments on the horizon. Evidence also supports other types of interventions, such as eviction prevention programs that help people avoid losing their housing in the first place.[9] Given that transition periods pose risks for a drug overdose, such interventions to prevent homelessness might also serve to prevent overdose. For individuals who are already homeless and need additional support services, permanent supportive housing that operates under a Housing First model offers apartments with voluntary services, such as case management, without imposing the precondition of sobriety. Multiple randomized clinical trials have found that supportive housing resolves homelessness, including for residents who continue to use drugs.[9] Housing First model programs already exist in many communities but, again, have not yet been brought to scale across the US.

Additional research and programming are needed to prevent overdose in supportive housing and other types of housing, such as co-location of safe consumption sites, on-site provision of harm-reduction supplies, and expansion of peer workers.[10] Other needed steps are outside the footprint of housing itself, such as ensuring a safe drug supply and decriminalization of drug use. Future work in overdose prevention should focus on forging partnerships and aligning systems, such as the homelessness, harm reduction, and treatment sectors, as well as full-scale adoption of evidence-based interventions to provide affordable housing and prevent homelessness. New housing funds are available to communities from the federal Coronavirus Aid, Relief, and Economic Security Act and the American Rescue Plan Act. These new opportunities should align with harm reduction and other services to reach unhoused individuals who are at risk for overdose.

The study by Fine et al[1] adds to a body of literature that convincingly demonstrates an association between homelessness and drug overdose death. Although further research will be helpful in elucidating points of intervention, it is already clear that housing is a fundamental human right and should be guaranteed for all. This work by Fine et al[1] should prompt not only an acknowledgment of the risk of overdose among people experiencing homelessness but also action that drastically reduces the number of people without stable, affordable housing.

ARTICLE INFORMATION

Published: January 7, 2022. doi:10.1001/jamanetworkopen.2021.42685

Open Access: This is an open access article distributed under the terms of the CC-BY License. © 2022 Doran KM et al. *JAMA Network Open*.

Downloaded from jamanetwork.com by guest on 12/07/2025

Corresponding Author: Kelly M. Doran, MD, MHS, Research Division, Department of Emergency Medicine, NYU School of Medicine, 227 E 30th St, New York, NY 10016 (kelly.doran@nyulangone.org).

Author Affiliations: Department of Emergency Medicine, New York University (NYU) School of Medicine, New York (Doran); Department of Population Health, NYU School of Medicine, New York (Doran); Department of Emergency Medicine, University of Washington School of Medicine, Seattle (Fockele); Corporation for Supportive Housing, Philadelphia, Pennsylvania (Maguire).

Conflict of Interest Disclosures: Dr Maguire reported serving as a board member of Prevention Point Philadelphia. No other disclosures were reported.

Disclaimer: The views expressed herein are those of the authors and do not reflect the official policy or position of their employers or other organizations with which the authors are affiliated.

REFERENCES

1. Fine DR, Dickins KA, Adams LD, et al. Drug overdose mortality among people experiencing homelessness, 2003 to 2018. *JAMA Netw Open*. 2022;5(1):e2142676. doi:10.1001/jamanetworkopen.2021.42676

2. Cawley CL, Kanzaria HK, Kushel M, Raven MC, Zevin B. Mortality among people experiencing homelessness in San Francisco 2016-2018. *J Gen Intern Med*. Published online April 9, 2021. doi:10.1007/s11606-021-06769-7

3. Los Angeles County Department of Public Health, Center for Health Impact Evaluation. Recent trends in mortality rates and causes of death among people experiencing homelessness in Los Angeles County. January 2021. Accessed November 8, 2021. http://www.publichealth.lacounty.gov/chie/reports/HomelessMortality2020_CHIEBrief_Final.pdf

4. Medical Examiner's Office, Department of Public Health. City of Philadelphia homeless death review report 2011-2015. Accessed November 3, 2021. https://www.phila.gov/media/20180418095811/HDR-Report-2011-2015-Deaths.pdf

5. Office of Community Planning and Development, US Department of Housing and Urban Development. The 2020 Annual Homeless Assessment Report (AHAR) to Congress. Part 1: point-in-time estimates of homelessness. January 2021. Accessed November 2021. https://www.huduser.gov/portal/datasets/ahar/2020-ahar-part-1-pit-estimates-of-homelessness-in-the-us.html

6. Eyrich-Garg KM, Cacciola JS, Carise D, Lynch KG, McLellan AT. Individual characteristics of the literally homeless, marginally housed, and impoverished in a US substance abuse treatment-seeking sample. *Soc Psychiatry Psychiatr Epidemiol*. 2008;43(10):831-842. doi:10.1007/s00127-008-0371-8

7. Colledge S, Peacock A, Leung J, et al. The prevalence of non-fatal overdose among people who inject drugs: A multi-stage systematic review and meta-analysis. *Int J Drug Policy*. 2019;73:172-184. doi:10.1016/j.drugpo.2019.07.030

8. Drake J, Charles C, Bourgeois JW, Daniel ES, Kwende M. Exploring the impact of the opioid epidemic in Black and Hispanic communities in the United States. *Drug Sci Policy Law*. Published online January 2020. doi:10.1177/2050324520940428

9. Shinn M, Khadduri J. *In the Midst of Plenty: Homelessness and What to Do About It*. Wiley Blackwell; 2020.

10. Bardwell G, Collins AB, McNeil R, Boyd J. Housing and overdose: an opportunity for the scale-up of overdose prevention interventions? *Harm Reduct J*. 2017;14(1):77. doi:10.1186/s12954-017-0203-9

Downloaded from jamanetwork.com by guest on 12/07/2025